A WORKING REFERENCE

The Product Manager's Guide to Healthcare Products

How healthcare actually works for the people who build software inside it — and where the Indian and American systems diverge in ways that change every product decision.

Scope. Stakeholders & money flow · product archetypes · the US system · the Indian system · regulation & compliance · data & interoperability · metrics · monetization · clinical risk · PM checklists · glossary.

India lens throughout US lens throughout

Current as of mid-2026. Treat dated facts — coverage figures, regulatory deadlines, scheme rules — as "true as of writing" and re-verify against primary sources before acting.

Contents

0 How to Use This Document

This is a working reference for product managers who build — or want to build — software inside healthcare. It assumes you already know how to do product. What it adds is the domain: the people, the money, the rules, and the two very different systems you are most likely to build for.

Who this is for

A PM moving into healthcare from another domain; a healthcare PM who owns one slice (say, a patient app or a CRM) and wants the whole map; or anyone preparing for healthcare-PM interviews who needs the system-level vocabulary that separates a confident answer from a vague one. You do not need a clinical or policy background.

The three lenses

Healthcare is not one market. Throughout this guide, three perspectives run in parallel: a **holistic lens** on how healthcare works anywhere, and two country lenses that are flagged inline — India and US. The same product idea — say, a teleconsultation app — has a completely different business model, regulatory burden, and buyer depending on which side of the world it lives in. The country lenses exist so you never confuse one set of rules for the other.

How the callouts work

PM LENS

What you specifically own or decide here — as opposed to clinical, legal, or engineering. If you read nothing else on a page, read these.

WATCH OUT

A costly, common mistake — or a place where India and the US behave so differently that an assumption carried from one will break in the other.

NOTE

Context, a definition, or a number worth keeping in your head.

ON ACCURACY

Healthcare facts move. Reimbursement codes, scheme coverage, regulatory thresholds, and interoperability mandates change yearly. Concepts in this guide are durable; specific figures and rules are dated to mid-2026. Before you make a real decision, verify the live version against the primary source (CMS, ONC, the FDA, MoHFW, NHA, ABDM, CDSCO, or the relevant regulator).

1 Why Healthcare Is Different for PMs

If you came from consumer or SaaS, almost every instinct needs adjusting. The person using your product is rarely the person paying for it; "ship fast and learn" collides with the fact that mistakes can harm people; and a regulator, a clinician, and a payer all sit between you and the outcome you want.

The seven structural truths

1. **The user, the buyer, and the payer are usually three different people.** A patient uses the app; a hospital or employer buys it; an insurer (or the government, or the patient's own pocket) pays for the care. You must satisfy all three, and they want different things. This is the single most important idea in the guide.
2. **Mistakes have a floor of consequence that consumer products don't.** A wrong dosage reminder, a missed critical lab flag, a mis-routed emergency — these are not churn events, they are harm events. "Move fast and break things" is not a value statement you can carry in unchanged.
3. **Regulation is upstream of product, not a compliance checkbox at the end.** Whether your software is a "medical device," what data you may store, and what claims you may make are determined before you design the feature. Retrofitting compliance is the most expensive way to learn this.
4. **Clinicians are a gate, not just a user.** Doctors and nurses can adopt or quietly kill a product. Workflow fit and time-saved matter more than elegance; a feature that adds thirty seconds per patient across a clinic's day will be abandoned regardless of how good it is.
5. **Outcomes are the real currency, and they are slow.** The thing healthcare actually wants to buy is better health at lower cost. Outcomes take months or years to show and are hard to attribute. Engagement metrics are a proxy you'll lean on — and a trap you can fall into (Section 9).
6. **Data is sensitive, fragmented, and rarely interoperable.** Health data is among the most protected categories of personal data anywhere, and it lives in dozens of incompatible systems. "Just integrate with the EHR" hides months of work (Section 8).
7. **Trust compounds and collapses asymmetrically.** It takes years to earn a clinician's or patient's trust and one bad incident to lose it. The brand risk of a healthcare product is categorically higher than a consumer app's.

> **PM LENS**
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> Your first job on any healthcare product is to draw the **stakeholder map** — who uses it, who decides to buy it, who pays for the underlying care, and who regulates it — before you write a single requirement. Most failed healthcare products optimized brilliantly for the user while ignoring the buyer or the payer.

2 Stakeholders & the Flow of Money

Healthcare's defining weirdness is that the money does not flow in a straight line from user to provider. Understanding who pays whom — and what each party is trying to maximize — explains almost every product constraint you'll hit.

The five core stakeholders

Stakeholder	Who they are	What they want
Patients	The people receiving care (your "users")	To get well, conveniently and affordably; to be heard and to trust the system
Providers	Doctors, nurses, hospitals, clinics, diagnostic labs, pharmacies	Better clinical outcomes, less administrative burden, and to get paid reliably
Payers	Insurers, employers, and governments who foot the bill	Lower total cost of care; fewer expensive events; verifiable value
Regulators	FDA, CMS/ONC (US); CDSCO, MoHFW, state bodies (India); plus data-protection authorities	Safety, efficacy, privacy, and fair markets
Pharma & medtech	Drug and device manufacturers	Adoption of their products; real-world evidence; access to prescribers and patients

The misaligned-incentive problem

In a normal market, the person who pays is the person who benefits, so they shop on price and quality. Healthcare breaks this. The patient consumes care but often doesn't pay directly; the provider recommends care but may be paid per service rendered (fee-for-service), which rewards volume, not health; the payer wants less care delivered, not more. These three pull in different directions, and your product sits in the middle.

> **NOTE — fee-for-service vs value-based**
>
> **Fee-for-service (FFS)** pays providers for each test, visit, or procedure — more activity, more revenue. **Value-based care (VBC)** ties payment to outcomes and total cost — keeping a population healthy is rewarded. The US is in a slow, contested migration from FFS toward VBC. India is overwhelmingly FFS and cash-driven. This single difference reshapes what a product can sell.

How the money actually flows

US. Money flows in a triangle: the patient (or their employer) pays premiums to an insurer; the patient receives care from a provider; the provider bills the insurer, who reimburses a negotiated rate; the patient covers the gap (deductible, copay, coinsurance). The complexity of getting the provider paid by the insurer is itself a massive industry — revenue cycle management (RCM) — and a fertile product area.

India. The dominant flow is far simpler and far harsher: the patient pays the provider **directly, out of pocket**, often in cash, at the point of care. As of the latest National Health Accounts, out-of-pocket expenditure was about 39% of total health spending — down sharply from ~64% a decade earlier, but still among the highest in the world. Insurance and government schemes are growing fast but cover a minority of total spend. This means in India, **the patient is frequently the payer**, which collapses the triangle and changes everything about pricing and trust.

WATCH OUT

A US-built "we save the payer money, so the payer buys us" model often has **no buyer in India**, because there frequently is no payer other than the patient — and patients won't pay a subscription to save a future hypothetical cost. Conversely, an India-built "patient pays cash per transaction" model struggles in the US, where the patient is insulated from price and shops on coverage, not cost.

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For any feature, ask the "follow-the-money" question explicitly: **Who is financially better off if this works, and do they control a budget?** If the answer is "the patient is healthier in three years," you have a noble feature and no clear buyer. Find the party whose money you save this quarter — that's your wedge.

3 Healthcare Product Archetypes

Most healthcare software falls into a handful of recognizable shapes. Knowing the archetype tells you the likely buyer, the likely regulatory exposure, and the likely failure mode before you've designed anything.

Archetype	What it does	Typical buyer	Key risk
EHR / EMR	System of record for clinical data — the chart	Hospitals, clinics	Workflow fit; switching cost; interoperability
Telemedicine	Remote consults (video/chat/async)	Patients, employers, providers	Regulation of remote prescribing; clinical quality
Marketplace / aggregator	Connects patients to doctors, labs, pharmacies, surgeries	Patients (demand) + providers (supply)	Two-sided liquidity; trust; leakage off-platform
RCM / billing	Gets the provider paid by the payer	Providers, hospital finance	Coding accuracy; payer rule changes
Digital therapeutics (DTx)	Software that treats a condition (e.g., diabetes, mental health)	Payers, employers, providers	Clinical evidence; regulatory (often a medical device)
Remote monitoring (RPM)	Devices + software tracking patients at home	Providers, payers	Alert fatigue; device accuracy; reimbursement
Diagnostics / imaging AI	Detects or flags disease from data/images	Hospitals, labs, providers	Almost always a regulated medical device
e-Pharmacy	Online medicine ordering & fulfillment	Patients	Drug regulation; cold chain; prescription validity
Care navigation / coordination	Guides patients to the right care; manages journeys	Employers, payers, providers	Proving it changed behavior & cost
Provider operations	CRM, scheduling, intake, comms, ops tooling	Clinics, hospitals	Integration; adoption by busy staff

NOTE — the regulation gradient

Archetypes sit on a spectrum of regulatory exposure. **Operations and marketplace** tools are usually lightly regulated (they don't make clinical claims). **Telemedicine and e-pharmacy** are regulated as practices (who may prescribe/dispense, and how). **DTx and diagnostic AI** are frequently regulated as medical devices — the heaviest burden — because the software itself influences a clinical decision. Know where you sit before you scope (Section 7).

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The fastest way to de-risk a new healthcare product is to start in a **low-regulation archetype that touches a high-regulation problem**. Many successful companies began as scheduling, navigation, or marketplace plays (light regulation, fast iteration) and earned the right — and the data — to move toward diagnostics or therapeutics later.

4 The US Healthcare System

The US system is privately delivered, mostly privately financed, fragmented across thousands of payers, and built on a complex machinery of insurance and reimbursement. It is the largest healthcare market in the world — roughly **18% of US GDP**, around **$4.9 trillion** a year — and almost none of that money moves without an insurance claim behind it.

Who pays: the payer landscape

Americans get coverage from a patchwork. Understanding which "bucket" a patient is in tells you who pays and what rules apply.

Coverage type	Who's covered	Who pays
Employer-sponsored (commercial)	Most working-age adults & families	Employer + employee premiums to a private insurer
Medicare	People 65+ and some with disabilities	Federal government
Medicaid	Low-income individuals & families	Federal + state governments (jointly)
ACA marketplace (individual)	People without employer/ government coverage	Individual premiums, often subsidized
Uninsured	~7–8% of the population	The patient, directly (or unpaid)

Roughly **92% of the population is insured**. That insulation matters: most US patients never see the true price of care, which is why "lower cost to the patient" is a weaker selling point than "covered by your plan."

> **NOTE — the alphabet of plan types**
>
> **HMO** (cheaper, must stay in-network, need referrals), **PPO** (pricier, more freedom, no referrals), and **HDHP** (high-deductible, often paired with an **HSA** tax-advantaged savings account). The plan type shapes patient behavior — a high deductible makes patients price-sensitive again, which is exactly where cash-pay and transparency products find a niche.

How providers get paid: coding & reimbursement

Every billable interaction is translated into standardized codes, bundled into a **claim**, and sent to the payer, who adjudicates and reimburses a negotiated amount. This is the beating heart of US healthcare ops, and the source of enormous product opportunity (and pain).

Code set	What it encodes
ICD-10-CM	Diagnoses — what's wrong with the patient
CPT	Procedures & services — what the provider did
HCPCS	Supplies, drugs, equipment, and non-physician services
DRG	Bundles an inpatient stay into one payment category
NPI	Unique ID for every provider

The lifecycle — eligibility check → service → coding → claim submission → adjudication → payment or **denial** → appeal — is **revenue cycle management (RCM)**. Claim denials are rampant, and reducing them is a direct, measurable dollar value: the rare healthcare pitch where you can name the buyer's savings precisely.

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If you build for US providers, you will eventually meet billing codes. A feature that helps a clinician document in a way that **codes correctly the first time** — capturing the right diagnosis specificity, avoiding a denial — has an immediate, quantifiable ROI. "Reduce denials by X%" is the kind of claim a hospital CFO will sign a check for.

Value-based care: the slow pivot

Policy is pushing the system from paying for volume to paying for value. Models like **ACOs** (Accountable Care Organizations) and bundled payments make providers financially responsible for a population's total cost and quality. Where VBC takes hold, the incentive flips: keeping people out of the hospital becomes profitable, which suddenly makes prevention, monitoring, and navigation products sellable. Adoption is real but uneven; most dollars still flow through fee-for-service.

The B2B2C reality

Because the patient rarely pays, most successful US digital-health companies sell to an intermediary who reaches the patient: an **employer** (who wants healthier, cheaper-to-insure employees), a **health plan** (who wants lower claims), or a **provider/health system**. The patient gets the product for free or cheap; someone upstream paid. This is the **B2B2C** model, and it's the default shape of US digital health.

WATCH OUT — the evidence bar

To sell to a US payer or employer, you typically need **clinical and economic evidence** — ideally a study showing your product improves an outcome or lowers cost. "Users love it" is not enough. Building the evidence base (pilots, peer-reviewed studies, actuarial analyses) is a multi-year product investment that founders from consumer backgrounds routinely underestimate.

NOTE — context worth carrying

Two themes dominate current US digital health: **GLP-1 medications** (the weight-loss/diabetes drug wave reshaping chronic-care economics) and **AI in clinical workflows** (ambient documentation, coding, triage). Both are payer-and-evidence games, not consumer-growth games.

5 The Indian Healthcare System

India's system is a study in contrasts: world-class private hospitals beside under-resourced public ones; cutting-edge digital public infrastructure beside cash-and-paper clinics; and a population that, for most care, still pays directly from its own pocket. It is large, fast-growing, price-sensitive, and structurally very different from the US.

Public vs private: a two-track system

Track	What it is	Reality
Public	Government hospitals & primary health centres; subsidized or free	Affordable but capacity-constrained; long waits; uneven quality outside cities
Private	Clinics, nursing homes, corporate hospital chains, diagnostics, pharmacies	Delivers the majority of outpatient care; ranges from premium to informal; mostly cash-pay

The private sector carries most of the load, especially for outpatient and specialist care. Public health spending remains low — government health expenditure is roughly **1.8% of GDP**, with total health spend a few points higher — which is why the burden has historically fallen on households.

The out-of-pocket reality

The defining fact of Indian healthcare: patients **pay directly**, at the point of care, for most of what they consume. Out-of-pocket expenditure was about **39% of total health spending** in the latest National Health Accounts — a major improvement from ~64% a decade earlier, driven by government schemes, but still very high by global standards. A single hospitalization can push a family into poverty, and this fear shapes patient behavior profoundly.

WATCH OUT

Because patients pay cash and feel every rupee, Indian healthcare products are **acutely price-sensitive and transaction-oriented**. Subscriptions, "save money later," and insurance-mediated value props are hard sells. What works: transparent prices, EMI/financing for big procedures, visible savings today, and trust signals (doctor credentials, reviews, hospital brand) that reduce the perceived risk of paying out of pocket.

The tier structure of demand

India is not one market. **Metro and Tier-1 cities** have insured, digitally fluent, English-comfortable users and premium private care. **Tier-2/3 towns and rural areas** are price-driven, often need vernacular-language and voice-first interfaces, have patchy connectivity, and rely heavily on local trust networks and informal providers. A product that wins in Bengaluru may be irrelevant in a Tier-3 town without rework.

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Language and modality are first-class product decisions in India, not localization afterthoughts. **Vernacular, voice-first, and low-bandwidth** design often determine reach more than features do. (This is exactly why Hindi-language voice interfaces for booking and triage are a live frontier — they meet users where they actually are.)

Insurance & government schemes

Coverage is growing from a low base. Private and employer health insurance is expanding in cities; **Ayushman Bharat (PM-JAY) —** the world's largest government health-assurance scheme — provides secondary and tertiary hospitalization cover to a large share of the population (around 40%, with senior citizens 70+ added recently). But schemes mostly cover hospitalization, not the day-to-day outpatient, diagnostic, and pharmacy spend where most money is actually spent out of pocket.

India's digital health public infrastructure

India has built ambitious "digital public goods" for health under the **Ayushman Bharat Digital Mission (ABDM)**. The key building blocks:

- **ABHA (Ayushman Bharat Health Account) —** a unique health ID for every citizen, the anchor for portable records.
- **Health Facility & Healthcare Professional Registries (HFR / HPR) —** verified directories of facilities and practitioners.
- **Health Information Exchange & Consent Manager (HIE-CM) —** consent-driven sharing of records between systems.
- **UHI (Unified Health Interface) —** an open network (UPI-style) intended to let any app discover and book services across providers.

The ambition mirrors what UPI did for payments: open, interoperable, consent-based rails that any product can build on. Adoption is uneven and evolving, but for a PM it represents both an integration opportunity and a direction of regulatory travel.

NOTE — the rules that govern Indian health products

Telemedicine Practice Guidelines (2020) legitimized and structured remote consults. **Drugs & Cosmetics rules** and evolving e-pharmacy regulation govern online medicine. The **Clinical Establishments Act** governs facilities (state-adopted unevenly). **CDSCO** regulates medical devices, including software as a medical device. And the **DPDP Act, 2023** is India's new data-protection law (Section 7).

WATCH OUT — fragmentation & informality

A large share of Indian healthcare is informal, paper-based, and unstandardized. Data is messy or absent; many providers don't use an EHR at all. Assume you will be **creating structured data, not integrating with it** — the opposite of the US "integrate with the incumbent EHR" problem.

6 India vs US — Quick Reference

The single page to return to. Every row here is a place where an assumption carried from one system will quietly break in the other.

Dimension	India	United States
Who pays for care	Mostly the **patient, out of pocket** (~39% of spend), often cash at point of care	Mostly **insurers** (employer, Medicare, Medicaid); patient pays deductibles/ copays
Health spend (% GDP)	~3–4% total; govt ~1.8%	~18% of GDP (~$4.9 trillion)
Price sensitivity	**Extreme** — every rupee is felt	**Muted** — patient insulated by insurance
Dominant buyer of B2B health products	Patient (B2C) or provider/hospital	Payer or employer (B2B2C)
Payment model	Overwhelmingly **fee-for-service / cash**	FFS, but migrating toward **value-based care**
Reimbursement coding	Minimal/emerging; not central to most products	**Central** — ICD/CPT/HCPCS, claims, RCM everywhere
Data & EHR maturity	Fragmented, often paper; you **create** data	Mature but siloed EHRs; you **integrate** with them
National digital rails	**ABDM** (ABHA, UHI, consent manager) — open, UPI-style	No single rail; **FHIR + TEFCA** interoperability mandates
Primary data-protection law	**DPDP Act, 2023**	**HIPAA** (+ state laws like CCPA)
Device/software regulator	**CDSCO** (under MDR 2017)	**FDA** (SaMD framework)
Telemedicine status	Legitimized by 2020 guidelines; widely used	Established; rules vary by state & payer
Language & access	**Vernacular, voice-first, low-bandwidth** often essential	English-default; literacy & access still matter
Evidence bar to sell	Trust, brand, price, convenience	**Clinical + economic evidence** for payers/employers
Winning wedge	Convenience, transparency, financing, trust at low price	Provable cost savings or outcome improvement to a budget-holder

PM LENS — the one-sentence test

Before porting any healthcare idea across these two systems, finish this sentence: "This works because ___ pays for it and ___ benefits." If you can't fill both blanks in the target country, the model doesn't transfer — it needs redesign, not translation.

7 Regulation & Compliance

In healthcare, two questions are settled before design begins: **Is my software a medical device?** and **What am I allowed to do with this data?** Get these wrong and you ship something illegal, unsellable, or both. This section is the orientation, not legal advice — bring a regulatory expert early.

When software becomes a medical device (SaMD)

Software as a Medical Device (SaMD) is software that performs a medical function on its own — diagnosing, screening, recommending treatment, or driving a clinical decision. The trigger is the **claim and the clinical influence**, not the technology. A symptom-checker that says "you may have X, see a doctor" is in murkier territory than one that "detects diabetic retinopathy" — the latter makes a diagnostic claim and is almost certainly regulated.

Usually NOT a device	Usually IS a device (SaMD)
Appointment booking, scheduling, CRM	Diagnoses or screens for a condition
General wellness & fitness tracking	Recommends or alters a treatment/dose
Administrative, billing, logistics tools	Analyzes images/signals to flag disease
Storing/displaying records (no interpretation)	Triages acuity in a way clinicians rely on

Aspect	US — FDA	India — CDSCO
Framework	FDA SaMD; risk classes I–III; pathways like 510(k), De Novo, PMA	Medical Device Rules 2017; risk classes A–D; software recognized as a device
Basis of scrutiny	Risk to patient if the software is wrong	Risk class drives licensing & testing
AI/ML angle	Active guidance on adaptive algorithms & "predetermined change control"	Evolving; expect tightening

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The cheapest regulatory strategy is **scoping your claims deliberately**. Wording the product as "decision support for a clinician who makes the call" vs "the product decides" can change your regulatory class — and your timeline by years. This is a product decision with a regulatory consequence, so own it; don't hand it to legal at the end.

Data protection: HIPAA vs DPDP

Aspect	HIPAA	DPDP Act, 2023
Scope	Protected Health Information (PHI) held by "covered entities" & their "business associates"	All digital personal data; health data is "sensitive" in spirit, with stricter expectations
Core duties	Privacy Rule, Security Rule, Breach Notification; need a **BAA** with vendors	Consent-based processing, purpose limitation, data-principal rights, breach reporting
Consent	Permitted uses for treatment/payment/operations; authorization for others	**Explicit, informed consent** is central; withdrawal must be easy
Penalties	Tiered civil & criminal penalties	Significant financial penalties under the Act

WATCH OUT

HIPAA is **not** a general "health privacy is handled" law — it only binds covered entities and their business associates, and a wellness app outside that chain may not be covered by it at all (which is its own risk). India's DPDP is broader in who it touches but newer and still operationalizing. **Do not assume one country's compliance posture satisfies the other.** If you handle data across borders, you need both, plus data-residency analysis.

NOTE — the other claims regulators

Beyond devices and data: **advertising & claims** ("clinically proven," "cures") are policed; **telemedicine** and **e-pharmacy** are regulated as practices (who may prescribe, dispense, and across what distance); and **controlled substances** have their own regime. Marketing copy is a compliance surface, not just a growth lever.

8 Data, Standards & Interoperability

"Just integrate with the EHR" is the sentence that has sunk more healthcare roadmaps than any other. Health data is sensitive, siloed, and encoded in standards that everyone implements slightly differently. Here's the map.

The standards that matter

Standard	What it's for
HL7 v2	The old workhorse for messaging between hospital systems — ubiquitous, cryptic, still everywhere
FHIR	The modern, API-friendly standard (resources over REST/JSON) — the direction the whole industry is moving
ICD-10 / ICD-11	Diagnosis coding (the "what's wrong")
SNOMED CT	A comprehensive clinical terminology for describing findings, procedures, etc.
LOINC	Codes for lab tests and observations
CPT / HCPCS	Procedure & billing codes (mostly US reimbursement)
DICOM	Medical imaging (radiology, etc.)

NOTE — FHIR is the verb

FHIR (Fast Healthcare Interoperability Resources) is what people mean today when they say "interoperability." It models health concepts as **resources** (Patient, Observation, Medication, Encounter…) exchanged over normal web APIs. If you're integrating with modern systems, you'll speak FHIR. But "FHIR-compliant" hides wide variation — every implementation has quirks.

Two very different interoperability worlds

US. Data is abundant but locked up inside competing EHR vendors. Federal mandates (the 21st Century Cures Act's information-blocking rules, FHIR API requirements, and the **TEFCA** nationwide exchange framework) are forcing the doors open. The product problem is **getting clean, permissioned access to data that already exists**, and avoiding being blocked.

India. Data is scarce and unstructured — much of it on paper or absent entirely. The national bet is **ABDM**: an open, consent-driven network (ABHA IDs, registries, consent manager, UHI) designed so that records become portable as they get digitized. The product problem is often **creating structured data in the first place**, then plugging into the emerging rails.

WATCH OUT — the integration tax

Every EHR/lab/pharmacy integration is a project with its own credentials, data quirks, sandbox, and review. Scope each one as **weeks-to-months, not days**. The demo that "reads the patient's chart" can hide a quarter of integration work. Build integration capacity as a roadmap line item, not a footnote.

PM LENS

Treat **data acquisition strategy** as core product strategy. In the US, ask: whose data, via which API, with what consent, and who can block me? In India, ask: how do I capture structured data at the source (voice, vernacular forms, OCR) and ride ABDM as it matures? The answer shapes your moat.

9 Metrics That Matter

Healthcare punishes the wrong metrics harder than most domains. Engagement can rise while outcomes fall; a buyer measures something completely different from what your user feels. Measure the thing your buyer actually pays for.

The three layers of healthcare metrics

Layer	Examples	Who cares
Product / engagement	Activation, retention, DAU, funnel conversion, task completion	You (proxy); necessary but not sufficient
Clinical / outcome	Adherence, blood-pressure control, HbA1c reduction, readmission rate, screening uptake	Clinicians, regulators, payers — the real prize
Economic	Cost per outcome, avoided ER visits/admissions, denial-rate reduction, total cost of care	Payers, employers, CFOs — who hold the budget

WATCH OUT — the vanity-metric trap

A mental-health app with soaring daily sessions might be making no one better — or worse, fostering dependence. A "we drove 10,000 consults" number is meaningless if those consults didn't change outcomes or cost. In healthcare, **high engagement that doesn't move a clinical or economic metric is a liability disguised as traction.** Always pair an engagement metric with the outcome it's supposed to cause.

What each buyer wants to see

- **A US payer/employer:** a credible line from your product to **lower total cost of care** — fewer admissions, ER visits, or complications — ideally validated by a study.
- **A provider/hospital: time saved** per clinician, throughput, denial reduction, and outcomes that affect their quality scores.
- **An Indian patient (B2C): convenience, transparent price, trust, and resolution** — did they get the care they needed, affordably, without anxiety?
- **A regulator: safety and efficacy** — does it work and does it avoid harm?

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Define your **"north-star outcome"** as the clinical or economic metric your buyer pays for, then instrument the engagement funnel that drives it as leading indicators. If your north star is an engagement number, you're optimizing a proxy and will eventually be found out by the person writing the check.

10 Monetization & Business Models

The hardest question in healthcare isn't "will people use it" — it's "who will pay, and how." The answer is almost entirely determined by which system you're in.

Model	How it works	Fits best
Pay-per-transaction (B2C)	Patient pays per consult, test, delivery, or procedure	India — cash-pay, price-sensitive, transactional
Procedure / care facilitation	Take rate on facilitating surgeries, diagnostics, treatments	India private care; marketplaces
B2B SaaS to providers	License software to clinics/hospitals (EHR, RCM, ops)	Both, esp. mid/large providers
B2B2C via payer/ employer	Sell to the budget-holder; reach patients free	US digital health default
Per-member-per-month (PMPM)	Payer pays a fixed fee per covered life	US value-based / population health
Risk / shared savings	You take financial risk on outcomes & share the savings	US VBC — advanced, high-trust
Pharma-funded	Manufacturers fund tools that drive adherence/RWE	Both — watch conflict-of-interest & claims rules
Subscription / membership	Recurring fee for ongoing care/access	US & Indian metros; hard in price-sensitive India

WATCH OUT

The most common monetization mistake is **importing the wrong model**. PMPM and shared-savings need a payer who manages risk — abundant in the US, rare in India. Pure cash-per-transaction needs price-sensitive self-payers — abundant in India, dampened in the US by insurance. Pick the model the system supports, not the one that worked in a deck from another country.

PM LENS

Pressure-test monetization with the **budget-holder question**: name the exact person/entity whose budget pays you, and the line item it comes from. "Patients will pay ₹X per booking out of pocket" and "the employer's benefits budget pays $Y PMPM to cut claims" are both valid — "people will see the value" is not a budget line.

11 Trust, Safety & Clinical Risk

This is the section that separates healthcare PMs from everyone else. When your product can influence a clinical decision, an edge case isn't a support ticket — it can be a harmed person. The discipline this demands should reshape how you scope, test, and ship.

The harm hierarchy

Not every healthcare feature carries the same stakes. Locate yours honestly:

Risk tier	Example	What it demands
Administrative	Booking, reminders, billing	Normal product rigor; protect data
Informational	Health content, record display	Accuracy, no implied diagnosis, clear disclaimers
Decision-influencing	Triage, symptom checks, dose reminders	Clinical review, conservative defaults, escalation paths
Decision-making	Diagnostic AI, autonomous recommendations	Regulatory clearance, validation, monitoring, human oversight

Principles for the risk-aware PM

- **Fail safe, not silent.** When unsure, your product should escalate to a human or advise care — never guess confidently. A symptom checker that under-triages a heart attack is a catastrophe; one that over-triages is merely annoying. **Design the asymmetry in.**
- **Keep a human in the loop where stakes are high.** "Decision support for a clinician who decides" is safer and often less regulated than "the software decides."
- **Edge cases are the product.** The rare presentation, the drug interaction, the pediatric vs adult dose — in healthcare these are not "later." Clinicians will judge you on them.
- **Involve clinical governance early.** A named clinical owner (a Medical Director or clinical lead) should sign off on anything that touches care. Build the review into your process, not around it.
- **Watch for automation bias & alert fatigue.** People over-trust confident software and tune out noisy alerts. Both are design problems you own.
- **Plan for the incident.** Have a way to detect, contain, communicate, and remediate a clinical-safety issue — before you need it.

WATCH OUT — AI's special burden

AI/LLM features amplify every risk here. They hallucinate, drift, and fail unpredictably on the long tail — exactly where healthcare lives. An AI scribe that invents a symptom, a chatbot that gives unsafe advice, a model that degrades on an under-represented group: these are clinical-safety events. **Constrain scope, ground outputs, keep humans accountable, and monitor in production.** "The model said so" is not a defense.

PM LENS

For every feature, write a one-line **"worst-case harm" statement**: "If this is wrong in the worst way, the consequence is ___." If that sentence describes physical harm, your testing, review, and rollout bar rises accordingly — and you should be able to defend it to a clinician and a regulator, not just a growth review.

12 PM Operating Checklists

The pages to return to. Adapt to your archetype and country; treat them as prompts, not gospel.

12.1 · Discovery & framing

- [ ] Drawn the full stakeholder map: user, buyer, payer, regulator, clinician.
- [ ] Answered "follow the money": who is financially better off if this works, and do they hold a budget?
- [ ] Named the country/system and confirmed the model fits it (India cash-pay vs US payer-mediated).
- [ ] Identified the product archetype and its default regulatory exposure.
- [ ] Defined the north-star outcome (clinical/economic), not just an engagement metric.
- [ ] Talked to real clinicians about workflow fit and time cost.

12.2 · Regulatory & compliance gate (before build)

- [ ] Determined whether the software is a medical device (SaMD) — and scoped claims deliberately.
- [ ] Identified the applicable regulator and pathway (FDA / CDSCO) if a device.
- [ ] Mapped the data-protection regime(s): HIPAA (+ BAAs) and/or DPDP; data residency for cross-border.
- [ ] Confirmed consent model, purpose limitation, retention, and deletion rights.
- [ ] Reviewed telemedicine / e-pharmacy / controlled-substance rules if relevant.
- [ ] Vetted marketing claims against advertising/claims rules.
- [ ] Engaged regulatory/legal expertise — not as a final sign-off, but as a design input.

12.3 · Clinical safety

- [ ] Located the feature on the harm hierarchy and written the worst-case-harm statement.
- [ ] Named a clinical owner / governance reviewer who signs off.
- [ ] Designed fail-safe behavior: escalate or advise care when uncertain.
- [ ] Defined human-in-the-loop checkpoints for high-stakes paths.
- [ ] Stress-tested edge cases (pediatric/elderly, comorbidities, rare presentations, language).
- [ ] For AI: scope constrained, outputs grounded, monitoring & rollback ready.
- [ ] Built an incident-response plan for a clinical-safety event.

12.4 · Data & integration

- [ ] Defined data acquisition strategy (integrate vs create; which APIs/standards).
- [ ] Scoped each EHR/lab/pharmacy integration realistically (weeks–months).
- [ ] Chosen standards (FHIR/HL7/coding) and validated against real partner data.
- [ ] In India: planned for ABDM rails (ABHA, consent manager, UHI) and structured-data capture.
- [ ] In US: checked information-blocking, access, and consent for the data you need.

12.5 · Launch & ongoing health

- [ ] Defined success as movement on the north-star outcome, with engagement as leading indicators.
- [ ] Set up an evidence plan (pilot → measurement → study) if selling to payers/employers.
- [ ] Instrumented clinical-safety and quality monitoring, not just product analytics.
- [ ] Established a feedback channel with clinicians/users for safety signals.
- [ ] Scheduled recurring review of regulatory/scheme/coding changes that affect the product.
- [ ] Confirmed accessibility: language, modality, bandwidth for the actual target users.

13 Glossary

The acronym soup, decoded. Country tags note where a term mostly applies.

Systems & payment

Payer — Whoever ultimately pays for care: an insurer, employer, or government. In India, frequently the patient themselves.

Provider — Anyone who delivers care: doctor, nurse, hospital, clinic, lab, pharmacy.

Fee-for-service (FFS) — Providers paid per service/test/visit. Rewards volume.

Value-based care (VBC) — Payment tied to outcomes and total cost. Rewards health, not activity. US

Out-of-pocket (OOP) — What patients pay directly. Dominant in India (~39% of total spend).

Medicare US — Federal coverage for people 65+ and some with disabilities.

Medicaid US — Joint federal-state coverage for low-income people.

ACA Affordable Care Act, US — Reform law; created the individual insurance marketplace and coverage rules.

HMO / PPO / HDHP US — Plan types trading off cost, network freedom, and deductible size; HDHPs often pair with an HSA.

ACO Accountable Care Organization, US — Provider group financially responsible for a population's cost & quality.

PMPM — Per-member-per-month; a fixed fee per covered life, common in US population-health deals.

Ayushman Bharat / PM-JAY India — Large government scheme covering secondary/tertiary hospitalization for a big share of the population.

Deductible / copay / coinsurance US — The shares of cost a patient pays before/alongside insurance.

Operations & billing

RCM — Revenue Cycle Management; the end-to-end process of getting a provider paid. US-heavy

Claim — The billing request a provider sends a payer for reimbursement.

Denial — A rejected claim; reducing denials has direct, measurable dollar value.

Prior authorization US — Payer pre-approval required before certain care is delivered/covered.

EHR / EMR — Electronic Health/Medical Record; the digital chart and system of record.

Coding & standards

ICD-10 / ICD-11 — International Classification of Diseases; diagnosis codes.

CPT US — Current Procedural Terminology; codes for procedures/services.

HCPCS US — Codes for supplies, drugs, equipment, non-physician services.

DRG US — Diagnosis-Related Group; bundles an inpatient stay into one payment.

NPI US — National Provider Identifier; unique provider ID.

HL7 v2 — Legacy messaging standard between hospital systems; still everywhere.

FHIR — Fast Healthcare Interoperability Resources; modern API-based standard. The industry's direction.

SNOMED CT — Comprehensive clinical terminology for findings, procedures, etc.

LOINC — Codes for lab tests and observations.

DICOM — Standard for medical imaging.

TEFCA US — Framework for nationwide health-information exchange.

India digital health

ABDM — Ayushman Bharat Digital Mission; India's national digital-health infrastructure program.

ABHA — Ayushman Bharat Health Account; a citizen's unique health ID.

UHI — Unified Health Interface; an open, UPI-style network for discovering and booking health services.

HFR / HPR — Health Facility Registry / Healthcare Professional Registry; verified directories.

Consent Manager (HIE-CM) — Consent-driven exchange of health records across systems.

Regulation & safety

SaMD — Software as a Medical Device; software that performs a medical function on its own.

FDA US — Regulates drugs and devices, including SaMD (risk classes I–III; pathways like 510(k), De Novo, PMA).

CDSCO India — Central Drugs Standard Control Organisation; regulates devices under the Medical Device Rules 2017 (classes A–D).

HIPAA US — Privacy & security law for PHI held by covered entities and business associates.

PHI US — Protected Health Information; health data covered by HIPAA.

BAA US — Business Associate Agreement; required contract for vendors handling PHI.

DPDP Act, 2023 India — Digital Personal Data Protection Act; India's data-protection law.

MoHFW India — Ministry of Health and Family Welfare.

NHA India — National Health Accounts (spending data) / National Health Authority (scheme implementation), per context.

Product categories

Telemedicine — Remote clinical care via video/chat/async.

DTx — Digital Therapeutics; software that treats a condition, usually evidence-backed and often regulated.

RPM — Remote Patient Monitoring; devices + software tracking patients at home.

RWE — Real-World Evidence; outcomes data gathered from real use, used to prove value.

B2B2C — Selling to a business (payer/employer/provider) that delivers your product to end consumers. The default US digital-health shape.

GLP-1 — A class of metabolic drugs (weight loss/diabetes) reshaping chronic-care economics. **US-prominent**

Closing note. This guide captures the durable structure of healthcare for product managers and the specific ways India and the US diverge. The structure ages slowly; the numbers, codes, schemes, and rules age fast. Use it to ask better questions — then verify the live specifics against primary sources before you decide. Current as of mid-2026.